

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Naltrexone 50mg tablets for the treatment of Alcohol Reduction and Dependence Management

Summary of NICE / NICE CKS Guidelines for Alcohol Reduction and Dependence Management

Overview

- Alcohol dependence and harmful drinking are common health conditions
- NICE CG115 provides comprehensive guidance on management
- Treatment should address both physical and psychological dependence

Assessment

- Use AUDIT (Alcohol Use Disorders Identification Test) score to assess severity
- Quantify units of alcohol consumed per week
- Assess degree of physical and psychological dependence
- Perform baseline liver function tests
- Assess patient motivation to reduce or abstain from alcohol

Management

- Brief interventions are first-line for mild-moderate dependence
- Specialist referral indicated for severe dependence requiring medically-assisted withdrawal
- Pharmacological support (naltrexone) combined with psychosocial interventions for moderate dependence
- Psychological support including CBT, family therapy, and peer support as appropriate

Red flags requiring urgent specialist referral

- Severe dependence requiring medically-assisted withdrawal
- Risk of delirium tremens or seizures
- Signs of Wernicke encephalopathy (confusion, ataxia, ophthalmoplegia)
- Acute liver failure or severe cirrhosis
- Severe comorbid psychiatric disorder

Patient Group Direction

for the supply/administration of

Naltrexone 50mg tablets

for the treatment of

Alcohol Reduction and Dependence Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adjunct to psychosocial interventions for alcohol reduction/abstinence in adults with mild-to-moderate alcohol dependence who are not requiring medically-assisted withdrawal.
Inclusion criteria	Age 18 years or over Diagnosed alcohol dependence (AUDIT score indicating harmful/dependent drinking) Demonstrated motivation to reduce alcohol consumption or abstain Not currently in acute alcohol withdrawal Abstinent or with significantly reduced alcohol consumption for at least 3-5 days before starting treatment Acceptable liver function (ALT and AST less than 3 times upper limit of normal) Informed consent given and documented
Exclusion criteria	Currently taking opioid medications or in acute opioid withdrawal (naltrexone is an opioid antagonist) Acute hepatitis or acute liver failure Liver function impairment with ALT or AST >5 times upper limit of normal

	Severe renal impairment (eGFR <30 mL/min) Pregnancy or breastfeeding Known hypersensitivity to naltrexone or any excipients Patients requiring medically-assisted alcohol withdrawal
Cautions	Hepatic impairment: monitor liver function regularly (at 2 weeks then monthly) Renal impairment: dose reduction may be needed for eGFR <50 mL/min Concurrent opioid medications: naltrexone will block opioid effects; careful consideration needed Patient should carry a medical alert card to inform healthcare providers that they take naltrexone (which blocks opioid analgesia in emergencies) Potential for depression or mood changes; counsel patient to report mood disturbances Avoid rapid withdrawal; taper dose if discontinuing
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Naltrexone 50mg tablets
Legal category	POM
Storage	Store below 25°C. Protect from light and moisture.
Route/method of administration	Oral. Swallow tablet whole with water. Take in the morning.
Dose and frequency	Day 1: 25mg once daily Day 2 onwards: 50mg once daily Take in the morning, with or without food
Quantity to be administered and/or supplied	Up to 28 tablets (28-day supply)
Maximum or minimum treatment period	Review at 2 weeks. If effective and well tolerated, continue for 6 months. Consider continuing for up to 12 months in some patients. Gradual dose reduction should be considered when discontinuing.
Adverse effects	Very common (>10%): Nausea, headache, insomnia Common (1-10%): Abdominal pain, vomiting, anxiety, nervousness, joint pain, muscle pain, dizziness, decreased appetite, fatigue

	Uncommon (0.1-1%): Depression, irritability, chest pain, palpitations Rare (<0.1%): Hepatotoxicity (dose-dependent, reversible with dose reduction or discontinuation)
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with naltrexone. Ensure patient understands it is an adjunct to psychological support, not a sole treatment.
Follow-up advice to be given to patient or carer	Do not take opioid medications while on naltrexone; contact GP or pharmacist before taking any painkillers. Carry your medical alert card at all times to inform healthcare professionals that you take naltrexone. Report any mood changes, depression, or suicidal thoughts immediately. Attend all follow-up appointments for liver function monitoring (blood tests at 2 weeks then monthly). Maintain engagement with psychological support (counselling, CBT, or group support). Do not suddenly stop naltrexone without consulting your healthcare provider.

	Inform all healthcare providers and dentists that you take naltrexone. Seek immediate medical attention if you experience severe abdominal pain, yellowing of skin or eyes, or dark urine.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Alcohol use disorders (NICE CG115)
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026
Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025